MedVoice AI

Complete Product Documentation

Vapi Migration · Component Inventory · Roadmap · GTM Strategy

Version: 1.0.0 — June 2026

Stage: Prototype (∼33% production-ready)

Contents

Executive Summary

MedVoice AI is a multi-tenant medical receptionist platform. The **product layer** (tenant config, calendar logic, lead capture, dashboards, billing) is yours. The **runtime layer** (real-time audio, STT, TTS, turn-taking) should be delegated to **Vapi** for fastest path to revenue.

Two integration wires: (1) assistant-request config at call start; (2) tool-calls + end-of-call-report during/after calls.

Current state: Web widget functional; phone scaffolded; Vapi not integrated; HIPAA not implemented.

Recommended path: L1 Phone MVP in 2 weeks → L2 first paying client in 6 weeks at \$1,500–2,500/mo.

Document index:

1. Vapi Integration Guide — exact implementation steps
2. Component Inventory — all modules with progress %
3. External Resources — provider stack and costs
4. Project Roadmap — L0 to L5 milestones
5. GTM Strategy — pricing, competitors, privileges

Chapter 1

Vapi Integration Guide

1.1 What Changes vs. What Stays

Full detail in standalone `01-vapi-integration.pdf`.

1.1.1 Drop

Browser phone orchestration, `voice.js` WebSocket scaffold, real-time media ownership.

1.1.2 Keep

`configs/`, calendar routes, leads, emergency, dashboard, SQLite, webhook pattern.

1.1.3 Build New

`server/routes/vapi.js` with `assistant-request`, `tool-calls`, `end-of-call-report` handlers.

1.1.4 Implementation Checklist

1. Vapi account + BYOK keys (Deepgram, OpenAI, ElevenLabs)
2. Twilio number imported to Vapi
3. Three webhook endpoints on MedVoice backend
4. Four Vapi tools mapped to existing calendar/lead APIs
5. Phone number → tenant mapping for multi-tenancy
6. Server-side webhook worker (replace browser outbox for phone)
7. E2E test: call → book → transcript in dashboard

Chapter 2

Component Inventory

See `02-component-inventory.tex`. Summary:

Surface	**Completion**	**Status**
Voice Widget (`js/`)	78%	Functional (web)
Express Backend (`server/`)	58%	Partial
React Dashboard	52%	Functional, no auth
Tests & DevOps	15%	Minimal
Overall	**33%**	**Not production-ready**

16 capabilities not started including Vapi, auth, billing, HIPAA, tenant admin, reschedule/cancel, SMS, after-hours.

Chapter 3

External Resources

See `03-external-resources.tex`. Recommended stack:

Twilio + Vapi + Deepgram + GPT-4o mini + ElevenLabs + Google Calendar + Stripe + AWS HIPAA

All-in cost: \$0.18–0.33/min. Price clinics at \$1,500–5,000/mo with 3–5× margin.

Chapter 4

Project Roadmap

See `04-project-roadmap.tex`.

Level	**Target**	**Key Deliverable**
L0	Now	Web widget prototype
L1	Week 2	Vapi phone MVP, 1 clinic
L2	Week 6	First paying client, HIPAA basics
L3	Month 4	10 clinics, self-serve onboarding
L4	Month 8	Analytics, EHR connectors
L5	Month 18	Enterprise, Epic integration

Chapter 5

Go-to-Market Strategy

See `05-gtm-strategy.tex`.

Market rate: Medical practices pay $1,200–25,000/mo depending on size.

MedVoice entry: $1,499/mo Starter; $999/mo Founding Practice pilot.

Seven privileges beyond price: Medical safety, appointment intelligence, after-hours revenue, zero turnover, multilingual, data ownership, transparent dashboard.

12-month target: 25 clinics, $75K MRR, $900K ARR.

Appendix A

Compilation Notes

Individual PDFs compile from `docs/latex/OX-*.tex`. Run `./compile.ps1` on Windows or `pdflatex` per file. Requires TeX Live or MiKTeX with `tikz`, `pgfplots`, `booktabs`.

Appendix B

Sources

- Vapi docs: https://docs.vapi.ai (pricing, tools, server URLs)
- LetsAskClaire: 2026 State of AI Receptionists in Medical Practice
- DeepCura: Best AI Medical Receptionist 2026 rankings
- MedVoice codebase: PRODUCT-CAPABILITY-MATRIX v1.1 (May 2026)